For the use of a Registered Medical Practitioner or a Hospital or a Laboratory or a Specialist only

Aceclofenac, Paracetamol & Chlorzoxazone Tablets

aceelo™

1. GENERIC NAME
Aceclofenac, Paracetamol & Chlorzoxazone Tablets

2. QUALITATIVE AND QUANTITATIVE COMPOSITION
Each film coated tablet contains :
Aceclofenac IP .....100 mg
Paracetamol IP.....325 mg
Chlorzoxazone USP .....250 mg
Colours : Sunset Yellow FCF & Titanium Dioxide IP

3. DOSAGE FORM AND STRENGTH
Film coated tablet

4. CLINICAL PARTICULARS
4.1 Therapeutic indications
It is indicated for the management of pain in Acute Musculoskeletal spasm

4.2 Posology and method of administration
Posology
The recommended dose is 1 tablet once daily or as directed by the Physician.
Method of administration: For oral administration only.

4.3 Contraindications
Known hypersensitivity to the active substance or to any of the excipients.
Active gastric or intestinal ulcer, bleeding or perforation.
History of gastrointestinal bleeding or perforation, related to previous NSAIDs therapy.
Active, or history of recurrent peptic ulcer/haemorrhage or perforation (two or more distinct episodes of proven ulceration or bleeding).
Last trimester of pregnancy
Severe hepatic, renal or cardiac failure.

4.4 Special warnings and precautions for use
Aceclofenac
Undesirable effects may be minimised by using the lowest effective dose for the shortest duration necessary to control symptoms. The use of Aceclofenac with concomitant NSAIDs including cyclooxygenase- 2 selective inhibitors should be avoided.
Elderly: The elderly has an increased frequency of adverse reactions to NSAIDs especially gastrointestinal bleeding and perforation which may be fatal.
Respiratory disorders: Caution is required if administered to patients suffering from, or with a previous history of, bronchial asthma since NSAIDs have been reported to precipitate bronchospasm in such patients.
Cardiovascular, Renal and Hepatic Impairment: The administration of an NSAID may cause a dose dependent reduction in prostaglandin formation and precipitate renal failure. Patients at greatest risk of this reaction are those with impaired renal function, cardiac impairment, liver dysfunction, those taking diuretics or recovering from major surgery, and the elderly. The importance of prostaglandins in maintaining renal blood flow should be considered in these patients.
Renal function should be monitored in these patients.
Renal: Patients with mild to moderate renal impairment should be kept under surveillance, since the use of NSAIDs may result in deterioration of renal function. The lowest effective dose should be used and renal function monitored regularly. Effects on renal function are usually reversible on withdrawal of Aceclofenac.
Hepatic: If abnormal liver function tests persist or worsen, clinical signs or symptoms consistent with liver disease develop or if other manifestations occur (eosinophilia, rash), Aceclofenac Tablets should be discontinued. Close medical surveillance is necessary in patients suffering from mild to moderate impairment of hepatic function. Hepatitis may occur without prodromal symptoms. Use of Aceclofenac in patients with hepatic porphyria may trigger an attack.
Paracetamol
Care is advised in the administration of paracetamol to patients with severe renal or severe hepatic impairment. The hazards of overdose are greater in those with non-cirrhotic alcoholic liver disease. Patients should be advised that paracetamol may cause severe skin reactions. If a skin reaction such as skin reddening, blisters, or rash occurs, they should stop use and seek medical assistance right away. Chlorzoxazone Serious (including fatal) hepatocellular toxicity has been reported rarely in patients receiving chlorzoxazone. The mechanism is unknown but appears to be idiosyncratic and unpredictable. Factors predisposing patients to this rare event are not known. Patients should be instructed to report early signs and/or symptoms of hepatotoxicity such as fever, rash, anorexia, nausea, vomiting, fatigue, right upper quadrant pain, dark urine, or jaundice.
Chlorzoxazone
It should be discontinued immediately and a physician consulted if any of these signs or symptoms develop. Chlorzoxazone use should also be discontinued if a patient develops abnormal liver enzymes (e.g., AST, ALT, alkaline phosphatase and bilirubin). The concomitant use of alcohol or other central nervous system depressants may have an additive effect.

4.5 Drugs interactions
Aceclofenac
Other analgesics including cyclooxygenase-2 selective inhibitors: Avoid concomitant use of two or more NSAIDs (including aspirin) as this may increase the risk of adverse effects including GI bleeding.
Anti-hypertensives: NSAIDs, may reduce the effect of activity antihypertensives. The risk of acute renal insufficiency which is usually reversible, may be increased in some patients with compromised renal function (e.g., dehydrated patients or elderly patients) when ACE-inhibitors or angiotensin II receptor antagonists are combined with NSAIDs. Therefore, the combination should be administered with caution, especially in the elderly. Patients should be adequately hydrated and consideration should be given to monitoring of renal function after initiation of concomitant therapy and periodically thereafter.
Diuretics: Aceclofenac, like other NSAIDs, may inhibit the activity of diuretics. Diuretics can increase the risk of nephrotoxicity of NSAIDs. Although it was not shown to affect blood pressure control when co-administered with bendrofluazide, interactions with other diuretics cannot be ruled out. When concomitant administration with potassium-sparing diuretics is employed, serum potassium should be monitored.
Cardiac glycosides like digoxin: NSAIDs may exacerbate cardiac failure, reduce GFR (glomerular filtration rate) and inhibit the renal clearance of glycosides, resulting in increased plasma glycoside levels. The combination should be avoided unless frequent monitoring of glycoside levels can be performed.
Lithium: Several NSAID drugs inhibit the renal clearance of lithium, resulting in increased serum concentrations of lithium. The combination should be avoided unless frequent monitoring of lithium can be performed.
Methotrexate: The possible interaction between NSAIDs and methotrexate should be born in mind also when low doses of methotrexate are used, especially in patients with decreased renal function. When combination therapy has to be used, the renal function should be monitored. Caution should be exercised if both an NSAID and methotrexate are administered within 24 hours of each other, since NSAIDs may increase plasma levels, resulting in increased toxicity.
Paracetamol
The anticoagulant effect of warfarin and other coumarins may be enhanced by prolonged regular daily use of paracetamol with increased risk of bleeding. The interaction is dose-dependent but may appear already at daily doses of 1.5-2g. Occasional doses have no significant effect. Concurrent administration of paracetamol and AZT (zidovudine) enhances the tendency to neutropenia. This medicinal product should therefore be co-administered with AZT only on medical advice. Concurrent intake of medicinal products that accelerate gastric emptying, such as metoclopramide, accelerates the absorption and onset of effect of paracetamol. Concurrent intake of medicinal products that slow gastric emptying can delay the absorption and onset of effect of paracetamol. The speed of absorption of paracetamol may be increased by metoclopramide or domperidone and absorption reduced by cholestyramine. Intake of cholestyramine and paracetamol should be separated by at least one hour. Probenecid reduces the clearance of paracetamol by almost 50%. Thus, the paracetamol dose may be halved during concomitant treatment. Alcohol abuse increases the risk for paracetamol toxicity.

4.6 Use in specific populations
Aceclofenac
Pregnancy:
There is no information on the use of Aceclofenac during pregnancy. Inhibition of prostaglandin synthesis may adversely affect the pregnancy and/or the embryo/fetal development. Data from epidemiological studies suggest an increased risk of miscarriage, cardiac malformation or gastroschisis after use of prostaglandin synthesis inhibitor in early pregnancy. The absolute risk for cardiovascular malformation was increased from less than 1%, up to approximately 1.5%. The risk is believed to increase with dose and duration of therapy. In animals, administration of a prostaglandin synthesis inhibitor has been shown to result in increased pre- and post-implantation loss and embryo-foetal lethality. In addition, increased incidences of various malformations, including cardiovascular, have been reported in animals given a prostaglandin synthesis inhibitor during the organogenetic period. During the first and second trimester of pregnancy, Aceclofenac should not be given unless clearly necessary. If Aceclofenac is used by a woman attempting to conceive, or during the first and second trimester of pregnancy, the dose should be kept as low and duration of treatment as short as possible. During the third trimester of pregnancy, all prostaglandin synthesis inhibitors may expose the foetus to:
- Cardiopulmonary toxicity (with premature closure of the ductus arteriosus and pulmonary hypertension);
- renal dysfunction, which may progress to renal failure with oligo-hydroamniosis; the mother and the neonate, at the end of pregnancy, to:
- Possible prolongation of bleeding time, an anti-aggregating effect which may occur even at very low doses.
- Inhibition of uterine contractions resulting in delayed or prolonged labour. Consequently, aceclofenac is contraindicated during the third trimester of pregnancy.
Lactation: There is no information on the secretion of Aceclofenac to breast milk, there was however no notable transfer of radio labelled (14C) Aceclofenac to the milk of lactating rats. The use of Aceclofenac Tablets should therefore be avoided in pregnancy and lactation unless the potential benefits to the other outweigh the possible risks to the foetus.
Fertility: The use of Aceclofenac tablets may impair female fertility and is not recommended in women attempting to conceive. In women who have difficulties conceiving or who are undergoing investigation of infertility withdrawal of Aceclofenac tablets should be considered

4.7 Effects on ability to drive and use machines
Patients experiencing visual disturbances, dizziness, vertigo, somnolence central nervous system disturbances, drowsiness or fatigue while taking aceclofenac, should refrain from driving or operate machinery

4.8 Undesirable effects
Aceclofenac
Fixed drug eruption (FDE) has been reported with Aceclofenac
Gastrointestinal: The most commonly-observed adverse events are gastrointestinal in nature. Peptic ulcers, perforation or GI bleeding, sometimes fatal, particularly in the elderly, may occur. Nausea, vomiting, diarrhoea, flatulence, constipation, dyspepsia, abdominal pain, melaena, haematemesis, ulcerative stomatitis, exacerbation of colitis and Crohn's disease have been reported following administration. Less frequently, gastritis has been observed. Pancreatitis has been reported very rarely.
Hypersensitivity: Hypersensitivity reactions have been reported following treatment with NSAIDs. These may consist of
(a) non-specific allergic reactions and anaphylaxis
(b) respiratory tract reactivity comprising asthma, aggravated asthma, bronchospasm or dyspnoea, or
(c) assorted skin disorders, including rashes of various types, pruritus, urticaria, purpura, angiodema and, more rarely exfoliative and bullous dermatoses (including epidermal necrolysis and erythema multiforme).
Cardiovascular and cerebrovascular: Oedema, hypertension and cardiac failure have been reported in association with NSAID treatment. Aceclofenac is both structurally related and metabolised to diclofenac for which a greater amount of clinical trial and epidemiological data consistently point towards an increased risk of general arterial thrombotic events (for example myocardial infarction or stroke, particularly at high doses or in long treatment). Epidemiological data has also found an increased risk of acute coronary syndrome and myocardial infarction associated with the use of aceclofenac. Exceptionally, occurrence of serious cutaneous and soft tissues infections complications during varicella has been reported in association with NSAID treatment Other adverse reactions reported less commonly include:
Renal:
interstitial nephritis
Neurological and special senses:
optic neuritis, reports of aseptic meningitis (especially in patients with existing auto-immune disorders, such as systemic lupus erythematosus, mixed connective tissue disease), with symptoms such as stiff neck, headache, nausea, vomiting, fever or disorientation, confusion, hallucinations, malaise, and drowsiness.
Haematological:
agranulocytosis, aplastic anaemia
Dermatological:
Bullous reactions including Stevens Johnson Syndrome and Toxic Epidermal Necrolysis (very rare). Photosensitivity.
Paracetamol
Fixed drug eruption (FDE) has been reported with Paracetamol
Adverse effects of paracetamol are rare. Very rare cases of serious skin reactions have been reported. There have been reports of blood dyscrasias including thrombocytopenia purpura, methaemoglobenaemia and agranulocytosis, but these were not necessarily causality related to paracetamol.
Chlorzoxazone
Occasional patients may develop gastrointestinal disturbances. It is possible in rare instances that chlorzoxazone may have been associated with gastrointestinal bleeding. Drowsiness, dizziness, lightheadedness, malaise, or overstimulation may be noted by an occasional patient. Rarely, allergic-type skin rashes, petechiae, or ecchymoses may develop during treatment. Angioneurotic edema or anaphylactic reactions are extremely rare. There is no evidence that the drug will cause renal damage. Rarely, a patient may note discoloration of the urine resulting from a phenolic metabolite of chlorzoxazone. This finding is of no known clinical significance.

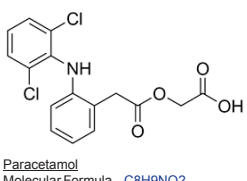
4.9 Overdose
Aceclofenac
a) Symptoms
Symptoms include headache, nausea, vomiting, epigastric pain, gastrointestinal irritation, gastrointestinal bleeding, rarely diarrhoea, disorientation, excitation, coma, drowsiness, dizziness, tinnitus, hypotension, respiratory depression, fainting, occasionally convulsions. In cases of significant poisoning acute renal failure and liver damage are possible.
b) Therapeutic measure:
Patients should be treated symptomatically as required. Within one hour of ingestion of a potentially toxic amount, activated charcoal should be considered. Alternatively, in adults, gastric lavage should be considered within one hour of ingestion of a potentially life-threatening overdose.
Specific therapies such as, dialysis or haemoperfusion are probable of no help in eliminating NSAIDs due to their high rate of protein binding and extensive metabolism. Good urine output should be ensured. Renal and liver function should be closely monitored. Patients should be observed for at least four hours after ingestion of potentially toxic amounts. In case of frequent or prolonged convulsions, patients should be treated with intravenous diazepam. Other measures may be indicated by the patient's clinical condition. Management of acute poisoning with oral aceclofenac essentially consists of supportive and symptomatic measures for complications such as hypotension, renal failure, convulsions, gastrointestinal irritation, and respiratory depression.
Paracetamol
Liver damage is possible in adults who have taken 10 g or more of paracetamol. Ingestion of 5g or more of paracetamol may lead to liver damage if the patient has risk factors.
Risk factors:
If the patient a, is on long term treatment with carbamazepine, phenobarbitone, phenytoin, primidone, rifampicin, St John's Wort or other drugs that induce liver enzymes. Or b, Regularly consumes ethanol in excess of recommended amounts. Or c, Is likely to be glutathione deplete e.g. eating disorders, cystic fibrosis, HIV infection, starvation, cachexia.
Symptoms:
Symptoms of paracetamol over dosage in the first 24 hours are pallor, nausea, vomiting, anorexia and abdominal pain. Liver damage may become apparent 12 to 48 hours after ingestion. Abnormalities of glucose metabolism and metabolic acidosis may occur. In severe 12 poisoning, hepatic failure may progress to encephalopathy, haemorrhage, hypoglycaemia, cerebral oedema, and death. Acute renal failure with acute tubular necrosis, strongly suggested by loin pain, haematuria and proteinuria, may develop even in the absence of severe liver damage. Cardiac arrhythmias and pancreatitis have been reported.
Management:
Immediate treatment is essential in the management of paracetamol overdose. Despite a lack of significant early symptoms, patients should be referred to hospital urgently for immediate medical attention. Symptoms may be limited to nausea or vomiting and may not reflect the severity of overdose or the risk of organ damage. Management should be in accordance with established treatment guidelines, see BNF overdose section. Treatment with activated charcoal should be considered if the overdose has been taken within 1 hour. Plasma paracetamol concentration should be measured at 4 hours or later after ingestion (earlier concentrations are unreliable). Treatment with N-acetylcysteine may be used up to 24 hours after ingestion of paracetamol, however, the maximum protective effect is obtained up to 8 hours post-ingestion. The effectiveness of the antidote declines sharply after this time. If required the patient should be given intravenous N-acetylcysteine, in line with the established dosage schedule. If vomiting is not a problem, oral methionine may be a suitable alternative for remote areas, outside hospital. Management of patients who present with serious hepatic dysfunction beyond 24h from ingestion should be discussed with the NPIS or a liver unit.
Chlorzoxazone
Symptoms:
Initially, gastrointestinal disturbances such as nausea, vomiting, or diarrhea together with drowsiness, dizziness, light-headedness or headache may occur. Early in the course there may be malaise or sluggishness followed by marked loss of muscle tone making voluntary movement impossible. The deep tendon reflexes may be decreased or absent. The sensorium remains intact, and there is no peripheral loss of sensation. Respiratory depression may occur with rapid, irregular respiration and intercostal substernal retraction. The blood pressure is lowered, but shock has not been observed.
Treatment:
Gastric lavage or induction of emesis should be carried out, followed by administration of activated charcoal. Thereafter, treatment is entirely supportive. If respirations are depressed, oxygen and artificial respiration should be employed and a patent airway assured by use of an oropharyngeal airway or endotracheal tube. Hypotension may be counteracted by use of dextran, plasma, concentrated albumin or a vasopressor agent such as norepinephrine. Cholinergic drugs or analeptic drugs are of no value and should not be used.

5. PHARMACOLOGICAL PROPERTIES
5.1 Pharmacodynamic properties
Aceclofenac is a non-steroidal agent with marked anti-inflammatory and analgesic properties. ATC code: M01A B16 the mode of action of aceclofenac is largely based on the inhibition to prostaglandin synthesis. Aceclofenac is a potent inhibitor of the enzyme cyclo-oxygenase, which is involved in the production of prostaglandins.
Paracetamol
Paracetamol is an antipyretic and analgesic. Paracetamol produces antipyresis through action on the hypothalamic heat regulation centre and analgesia by elevation of the pain threshold. Paracetamol has analgesic and antipyretic actions similar to aspirin, but it has no useful anti-inflammatory properties. Paracetamol produces its analgesic effect from the inhibition of prostaglandin synthesis. Prostoglandins appear to sensitize pain receptors to mechanical stimulation or to other chemical mediators. Paracetamol lowers the body temperature in patients with fever but rarely lowers normal body temperature. This is again due to the inhibiton of synthesis and release of prostoglandins. The drug also acts on the hypothalamus to produce antipyresis; heat dissipation is increased as a result of vasodilation and increased peripheral blood flow. Paracetamol is generally well tolerated by patients hypersensitive to acetylsalicylic acid.
Chlorzoxazone
Chlorzoxazone is a centrally acting agent for painful musculoskeletal conditions. Data available from animal experiments as well as human study indicate that chlorzoxazone acts primarily at the level of the spinal cord and subcortical areas of the brain where it inhibits multisynaptic reflex arcs involved in producing and maintaining skeletal muscle spasm of varied etiology. The clinical result is a reduction of the skeletal muscle spasm with relief of pain and increased mobility of the involved muscles.

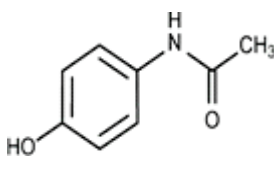
5.2 Pharmacokinetic properties
Aceclofenac
Following oral administration, aceclofenac is rapidly and completely absorbed as unchanged drug. Peak plasma concentrations are reached approximately 1.25 to 3.00 hours after ingestion. Aceclofenac penetrates the synovial fluid, where its concentrations reach approximately 57% of those of plasma. The volume of distribution is approximately 25 l. The plasma half-life is about 4 hours. Aceclofenac is largely protein bound (> 99%). Aceclofenac circulates primarily in unchanged drug form. The major metabolite detected in plasma is 4'-hydroxyaceclofenac. Approximately two thirds of the administered dose is excreted in the urine, mainly in the form of hydroxymetabolites. No alterations in the pharmacokinetics of aceclofenac have been detected in elderly patients.
Paracetamol
Paracetamol is rapidly and almost completely absorbed from the alimentary tract. Peak plasma concentrations occur after 30 minutes to two hours following oral dosing. Paracetamol is metabolised in the liver and excreted in the urine mainly as the glucoronide and sulphate conjugates with about 10% as glutathione conjugates. Less than 5% is excreted as unchanged paracetamol. The elimination half-life varies from about 1 – 4 hours. Plasma protein binding is negligible at usual therapeutic concentrations, although this is dose dependent.
Chlorzoxazone
Blood levels of chlorzoxazone can be detected in people during the first 30 minutes and peak levels may be reached, in the majority of the subjects, in about 1 to 2 hours after oral administration of chlorzoxazone. Chlorzoxazone is rapidly metabolized and is excreted in the urine, primarily in a conjugated form as the glucuronide. Less than one percent of a dose of chlorzoxazone is excreted unchanged in the urine in 24 hours.

6. NONCLINICAL PROPERTIES
Aceclofenac
The results from preclinical studies conducted with aceclofenac are consistent with those expected for NSAIDs. The principal target organ was the gastro-intestinal tract. No unexpected findings were recorded. Aceclofenac was not considered to have any mutagenic activity in three in vitro studies and an in vivo study in the mouse. Aceclofenac was not found to be carcinogenic in either the mouse or rat. Animal studies indicate that there was no evidence of teratogenesis in rats although the systemic exposure was low and in rabbits, treatment with aceclofenac (10 mg/kg/day) resulted in a series of morphological changes in some fetuses.
Paracetamol
In animal experiments regarding acute, subchronic and chronic toxicity of paracetamol in rats and mice, gastrointestinal lesions, blood count changes, degeneration of liver and renal parenchyma, even necroses were observed. The causes for these changes are attributed to the mechanism of action on the one hand and on the other to the metabolism of paracetamol. Extensive investigations showed no evidence of a relevant genotoxic risk of paracetamol at therapeutic, i.e. non-toxic doses. Long-term studies in rats and mice yielded no evidence on relevant tumorigenic effects at non-hepatotoxic dosages of paracetamol. Paracetamol passes through the placenta. Animal studies yield no evidence on reproductive toxicity.

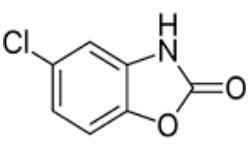
7. DESCRIPTION
This product (tablet) contains: Aceclofenac, paracetamol & chlorzoxazone as active ingredient and It is indicated for the management of pain in Acute Musculoskeletal spasm
Aceclofenac
Molecular Formula– $C_{16}H_{13}Cl_2NO_4$
Molecular Weight – 354.2 g/mol
Chemical Structure



Paracetamol
Molecular Formula – C8H9NO2
Molecular Weight – 151.16 g/mol
Chemical Structure



Chlorzoxazone
Molecular Formula – C7H4ClNO2
Molecular Weight – 169.56 g/mol
Chemical Structure



8. PHARMACEUTICAL PARTICULARS
8.1 Incompatibilities
Not applicable.
8.2 Shelf-life
as per carton
8.3 Packaging information
20x10 Tablets
8.4 Storage and handing instructions
Store protected from light & moisture, at a temperature not exceeding 30°C.
Keep all medicines out of reach of children.
Warning : Not for veterinary use.

9. PATIENT COUNSELLING INFORMATION
Read all of this leaflet carefully before you start taking this medicine because it contains important information for you.
Keep this leaflet. You may need to read it again.
If you have any further questions, ask your doctor, pharmacist or nurse.
This medicine has been prescribed for you only. Do not pass it on to others. It may harm them, even if their signs of illness are the same as yours.
If you get any side effects, talk to your doctor, pharmacist or nurse. This includes any possible side effects not listed in this leaflet.

10. DETAILS OF MANUFACTURER
Pure & Cure Healthcare Pvt. Ltd.
(A subsidiary of Akums Drugs & Pharmaceuticals Ltd.)
Plot No. 26A, 27-30, Sector-8A, I.I.E., SIDCUL,
Ranipur, Haridwar-249 403, (Uttarakhand).

11. DETAILS OF PERMISSION OR LICENCE NUMBER WITH DATE
31/UA/2013 Date: 30/06/2014

12. DATE OF REVISION
09-10-2026

AUSPHARMA™

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